BASELINE → TREATMENT → FOLLOW-UP

Checks across your colorectal treatment

There is no fixed NCCP scan or tumour-marker interval in this regimen. The team individualises imaging, markers and extra tests.

Check	Before starting	Infusion visits	Between visits	Course review	After treatment
Blood count, kidneys, liver and glucose	Baseline blood tests	Before every treatment cycle	Extra tests for new symptoms or abnormalities	Clinical review before restart after a hold	Follow abnormal results to resolution
Thyroid and other hormones	Baseline thyroid function	Check thyroid timetable	Thyroid every 3–6 weeks; cortisol if indicated	Confirm any hormone replacement plan	Hormone treatment may continue long term
Symptoms and immune inflammation	Discuss autoimmune disease, infection, medicines and previous immunotherapy	Review new symptoms before infusion	Report symptoms immediately; do not wait for the next cycle	Pauses or stopping are clinician decisions	Immune effects can occur after the last dose
Cancer response and duration	Confirm validated MSI-H/dMMR result and first-line setting	Agree the response assessment plan	Scans and markers as directed by the consultant	Individual review, including possible stop after sustained response at 24 months	Agree follow-up and the contact pathway

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00558 v13 official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab 400 mg · colorectal: treatment and checks

First-line metastatic MSI-H / dMMR colorectal cancer

Pembrolizumab 400 mg is given over about 30 minutes on Day 1 every 6 weeks (42 days). The visit takes longer for assessment and preparation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Infusion visits	Between visits	Course review	After treatment
Blood count, kidneys, liver and glucose NCCP 00558	Baseline blood tests	Before every treatment cycle	Extra tests for new symptoms or abnormalities	Clinical review before restart after a hold	Follow abnormal results to resolution
Thyroid and other hormones NCCP 00558	Baseline thyroid function	Check thyroid timetable	Thyroid every 3–6 weeks; cortisol if indicated	Confirm any hormone replacement plan	Hormone treatment may continue long term
Symptoms and immune inflammation NCCP 00558	Discuss autoimmune disease, infection, medicines and previous immunotherapy	Review new symptoms before infusion	Report symptoms immediately; do not wait for the next cycle	Pauses or stopping are clinician decisions	Immune effects can occur after the last dose
Cancer response and duration NCCP 00558	Confirm validated MSI-H/dMMR result and first-line setting	Agree the response assessment plan	Scans and markers as directed by the consultant	Individual review, including possible stop after sustained response at 24 months	Agree follow-up and the contact pathway

What each check is for

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Pembrolizumab every 6 weeks

One immunotherapy medicine

Pembrolizumab aims to control bowel cancer that has spread and has a particular DNA-repair feature, called MSI-H or dMMR. Your team discusses likely benefit, uncertainty and alternatives for you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00558

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35